

01 THE SOLUTION

Personalized care and wellness for every veteran, and their family, delivered through *their phone*.

InPursuit Health unifies a veteran's complete health story across every source, reasons over it, and delivers proactive, personalized care and wellness directly to the veteran and their household. Not another portal to log into. The infrastructure the country has been trying to build, placed in the hands of the people who earned it.



THE FEDERAL FOUNDATION IS FUNDED AND IN LAW

Congress has enacted the most consequential run of veterans legislation in a generation: the VA MISSION Act (Public Law 115-182), which opened community care to millions of veterans; the Forever GI Bill (Public Law 115-48); and the VA Accountability and Whistleblower Protection Act (Public Law 115-41), alongside record VA budgets and nationwide telehealth authority.

That foundation is funded and operating now. FY26 VA funding stands at a record \$445 billion, up 12.4% over FY25. \$37 billion is committed to rebuilding the records infrastructure at every VA medical center. The legacy portals consolidate into a single front door on September 30, 2026, and VHA's new Digital Health Office has asked the market, in writing, for a unified interoperability framework with real-time data exchange (RFI 36C10G26Q0072).

InPursuit Veterans, LLC, a majority veteran-owned company led by a USMC veteran, is built for this moment. The technology it delivers is already proven inside the CMS Health Tech Ecosystem, recognized across all three patient-facing categories: Kill the Clipboard, Conversational AI, and Chronic Care. The InPursuit Health App was independently reviewed and awarded the DiMe Seal, the quality credential CMS selected for its Medicare App Library, valid July 14, 2026 through July 13, 2027.

A proven capability, a funded federal priority, and 7 million veterans waiting on the outcome.

☰ **One complete health record:** VA, DoD, Medicare, TRICARE, and community care unified into a single living record. No portals, no PDFs assembled by hand.

📁 **Real-time insurance visibility:** coverage, benefits, and what care will cost, before the visit, not after the bill.

🕒 **Wearables flow in:** watches, rings, and home monitors integrate into the same record their care runs on.

📦 **Lower-cost medications:** prescription price visibility and lower-cost options, surfaced automatically.

🕒 **Watched over, not just stored:** chronic conditions and cardiac risk surfaced early, where care matters most.

👤 **The whole household in:** built around the caregiver spouse and the family that supports them. The family is a member, not an afterthought.

📁 **Conversational AI that knows them:** answers personalized to the Veteran's own health and wellness profile, grounded in their record, not generic advice.

📶 **Vitals from the phone's camera:** contactless measurement built on FDA-cleared components.

📅 **Easier scheduling:** appointments across VA and community providers, managed in one place.

★ **Real value from their data:** offers and cashback routed directly to the Veteran, not to a layer between them and their care.

02

THE GAP

Tens of billions spent. The veteran still stitches it together by hand.

The tools a veteran has access to today let them look at fragments of their own record. None of them unify it, reason over it, or reach the family. This is the gap, program by program.

\$37B

Current lifecycle estimate for VA EHR modernization, up from a \$10B contract. Earlier independent estimates ran near \$50B.

58%

Of VA staff believe the modernized system increased patient safety risks, per federal watchdog review.

13%

Of VA staff believe the new system made the VA as efficient as possible.

VA Blue Button / My HealthVet	Records export. View, print, or download your own VA records as a file. No intelligence, no coordination, and VA data only.
VA Health and Benefits app	Transactional. Appointments, claims, and prescriptions. Real adoption, but VA-only, single-person, and no analytics over the data.
MHS Genesis Patient Portal	A separate silo. DoD records, appointments, and messaging, limited to military treatment facilities. It does not talk to the VA side.
The dual-eligible reality	Two logins. A veteran with TRICARE is told to use both portals separately. Neither touches their Medicare data, community providers, or family.

THE SIZE OF THE PRIZE

The VA spends roughly **\$14,750 per veteran patient each year** across its medical-care programs. InPursuit's established commercial rate is **\$89.99 per month per household**. Offered to the VA at \$50 per veteran per month, full deployment to all 7 million veterans in VA care is roughly a **\$4.2 billion annual contract**, under 3% of the VA's annual medical-care budget. And the system is built to save the government billions more by eliminating wasted, duplicated healthcare spending.

7M

Veterans in VA care, every one addressable under a single contract vehicle.

\$4.2B

Annual contract value at \$50 per veteran per month, full deployment.

<3%

Of the VA's annual medical-care budget, against \$14,750 already spent per patient.

Today, a veteran has fragments. With InPursuit, a veteran has their whole health story, working for them.

03 THE CONTRACTING PATH

One authority. Any scope. From a measured pilot to *every veteran*.

InPursuit Veterans, LLC can be awarded a sole-source contract to deploy the InPursuit system to all 7 million veterans, at any scale, under a single federal authority. The basis is straightforward: the system is proprietary, InPursuit Veterans holds its exclusive VA distribution license, and the law provides for exactly that circumstance. No dollar ceiling applies.

The authority FAR 6.302-1 • 41 U.S.C. 3304(A)(1)

Federal law permits award without full and open competition when only one responsible source can meet the requirement. The contracting officer documents the basis in a written justification and approval (FAR 6.303, 6.304), made publicly available (FAR 6.305). The authority carries no dollar limit. The identical basis supports a measured pilot and full deployment to all 7 million veterans.

Why InPursuit Veterans is the only source

InPursuit Health is the original manufacturer (OEM) of both the orchestration infrastructure and the application, a proprietary, end-to-end system. InPursuit Veterans, LLC holds the exclusive license to furnish that system to the VA. No other vendor can furnish, resell, or replicate it. Where the requirement is the InPursuit system, InPursuit Veterans is, by definition, the only responsible source.

Fair and reasonable price, documented

The contracting officer must also find the price fair and reasonable. The system is offered below its established commercial rate of \$89.99 per month per household, and at a fraction of the roughly \$14,750 the VA already spends per veteran patient each year. Full deployment lands under 3% of the VA's annual medical-care budget.

Veteran-owned priority 38 U.S.C. 8127

InPursuit Veterans, LLC is majority veteran-owned and led full-time by a USMC veteran, with SBA VetCert certification decision expected August 2026. Once certified, it also carries the VA's Veterans First priority, which strengthens the case but is not required for the sole-source authority above.

WHY THIS MATTERS FOR INVESTORS

The same infrastructure already recognized inside the CMS Health Tech Ecosystem serves the VA opportunity without a rebuild. InPursuit Health owns the system; InPursuit Veterans, LLC, its majority veteran-owned affiliate, carries it to the VA under an exclusive, fair-market-value license, converting a \$4.2 billion annual opportunity from a bid into a documented, lawful, non-competitive path. Every VA deployment flows through that license, so the value of the federal channel accrues to the technology InPursuit Health investors own. One authority covers every scale of engagement, so a pilot win and full deployment rest on the identical legal basis.

[See the Veterans First Initiative](#)inpursuithealth.com/veterans-first